

Input vs. Output	• What are users inputting (for example, audio, text, images)? • What are the outputs or labels your model will predict or generate?	Users will input text-based health questions. The model will generate concise, evidence-based text responses that provide appropriate public-health guidance, safety information, and triage cues when urgent medical attention may be needed
Classification problem	• What is the classification task, if any? • What categories or classes are you working with, and why?	The project is primarily a medical Q&A task rather than a classification problem. However, we may classify questions by topic, population, and care setting to improve retrieval. Topics include maternal health, chronic disease, infectious disease, medication safety, nutrition, vaccination, emergency triage, and mental-health basics.
Labeling	• Where do your labels come from—domain experts, community members, crowd workers, AI-generated sources? • How will you check for consistency and accuracy?	Labels and reference answers will come primarily from the provided benchmark dataset on kaggle and trusted public-health sources. Where additional data is created, it will be reviewed against authoritative guidance from organizations such as WHO, UNICEF, and relevant Nigerian health authorities.
Data creation	• How will you collect your data? • Will it come from direct sources (like interviews or recordings), or will	Our data will primarily come from verified public-health sources such as WHO, UNICEF, CDC, and relevant Nigerian health authorities.

	you synthesize data in low-resource settings (e.g., for under-resourced languages)?	We will collect publicly available health guidelines, factsheets, articles, and reports that correspond to the benchmark topics across all regions primarily for African settings
Web collection ethics	• Do you plan to source data from the web? • If yes, which sites or platforms? • What tools will you use - AI tools, human workers, or both? • Have you addressed issues of consent, copyright, and privacy?	Yes. We plan to use publicly available resources from trusted organizations such as WHO, UNICEF, CDC, and Nigerian public-health authorities. We will prioritize documents that permit reuse or are openly available for research, retain source attribution, and respect the copyright and licensing conditions of each source. We will not collect private or personally identifiable health information, and human review will be used to verify important medical content.
Bias and representation	• Who might be left out of your dataset? • How will you ensure that the dataset reflects the diversity of the community and includes underrepresented voices?	A dataset based only on global health sources may overlook the realities, language, healthcare access, and cultural context of African communities. We will therefore include relevant Nigerian and African public-health guidance alongside global sources and consider different populations, including pregnant women, infants, children, and adults. We will also identify gaps in geographic, socioeconomic, and population representation rather than assuming one guideline fits everyone.

Values and community alignment	• How does your dataset design reflect the values you identified in 01 Build Your Own Small Language Model • How will your community benefit from this dataset, and how will you ensure accountability to them?	Our dataset design is guided by safety, accuracy, fairness, inclusivity, transparency, accessibility, and respect for human dignity. We will prioritize trustworthy sources, clearly distinguish health education from professional medical care, and ensure that urgent situations include appropriate guidance to seek medical attention. By providing accessible and evidence-based information, we hope to help people make safer health decisions while remaining accountable through source attribution, human review, and continuous evaluation.
--	--	--